



Osteopenia And Osteoporosis... Oh My! What Do I Do Now?

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It's Personal

A few months ago, I requested a dual-energy X-ray absorptiometry (DEXA) scan be performed and when the results came back indicating that I had mild osteopenia... *I was surprised and thought "Hey! I am doing this on a 'whim' as a baseline! And what about all those women who wait until they are 65 to have it done!"* You see, I am post-menopausal but not yet age 65 (I have several more years to go!).

For those not familiar with the term DEXA, it is the 'gold standard' for measuring bone density, which is an assessment of bone mineral content at key sites such as the lumbar spine, hip and forearm. I will cover DEXA scans in more detail a little later... Anyway, I talked to multiple friends and found out some of them also have osteopenia and a few already have osteoporosis... *and guess what!!! They are all around my age!!* This got me thinking that **'a flag needs to be raised'** to alert mature people (particularly post-menopausal women; peri-menopausal too!) **to start getting bone density scans at an earlier age!**

Osteopenia and Osteoporosis... What's the Difference?

Osteopenia (also called "low bone mass") is a condition of mildly reduced bone density that serves as a warning sign for bone loss. It is medically characterized as low bone density (BMD) that falls between T-Score of -1.0 and -2.5 standard deviation below the average peak bone density of a healthy 30-year-old person. NOTE: A T-score of -1.0 or above indicates normal bone density.

Osteoporosis, on the other hand, is a more advanced, serious condition characterized by weak, porous, brittle bones and a high risk of fractures. These common fracture sites include the hip, spine and wrist. A T-score of less than -2.5 indicates progression to osteoporosis and a significant increase in fracture risks.

Through this research I learned that osteopenia and osteoporosis are known as the 'Silent Diseases' as they progress silently without obvious symptoms... well, until a fracture occurs. **"Osteopenia affects an estimated 44 million Americans, representing a critical stage in bone health that often goes undetected until significant density loss has occurred."** *What a scary thought... Let me repeat that, usually undetected until significant density loss has occurred!!* An osteopenia diagnosis should serve as an **'early warning sign'** that your bones are weakening but have not deteriorated to the more severe osteoporosis... meaning there is still time to slow down its progression! [4]

So, let's take a deeper dive, 'see' what's going on in there, and determine through Functional Nutrition and lifestyle changes (and that includes exercise... Yikes!!), how to slow down progression and nourish our bones!! *(Please note this paper is written with osteopenia in mind*



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and slowing down progression; however, nutrition/lifestyle changes described for osteopenia will also help with osteoporosis (unless otherwise noted.)

What Causes Osteopenia and Osteoporosis?

In general, both osteopenia and osteoporosis are driven by aging, hormonal changes (especially post-menopause), poor diet, gut dysfunction, smoking, excess alcohol, sedentary lifestyle and certain medications such as glucocorticoids. The hormone imbalances that tend to contribute to osteopenia and osteoporosis include low estrogen in women, low testosterone in men (*Yes men...you too can get the 'osteos'!!*), thyroid dysfunction and elevated cortisol. Nutritional deficiencies include Calcium, Vitamin C, Vitamin D, Vitamin K2, Magnesium, and Protein to name a few. For women in particular, estrogen helps to maintain the balance of bone formation and breakdown, and due to post-menopausal decrease in estrogen in the body there is an acceleration in bone density loss rate. Chronic inflammation can also contribute significantly to bone density loss over time.

Glucocorticoids... What Are They?

I am glad you asked... Let's find out! Our bodies naturally produce these steroidal hormones from the adrenal cortex. In the body, glucocorticoids play a key role in glucose, protein and fat metabolism with the essential glucocorticoid being cortisol. [2] They are also potent immune suppressors that reduce the innate immune function's pro-inflammatory response and suppress adaptive immune function. When taken as a medication, glucocorticoid or corticosteroids are synthetic therapeutics that are similar to cortisol used to manage various inflammation symptoms. The most common types include: Prednisone, Cortisone, Hydrocortisone, and Methylprednisolone.

Are There Any Signs? Come On Body...Talk to me!!

In this case, Silence IS NOT Golden... Or are you screaming and I can't hear you! Subtle warning signs of developing osteopenia or osteoporosis often appear gradually and can be easy to overlook. (*No wonder I can't hear anything!*) Over time, a person may notice a **loss of height**, typically due to compression of the vertebrae, or subtle **changes in posture**, such as a forward head position or a rounding of the upper back (kyphosis). [8,9] Clothing may fit differently, or the rib cage may appear closer to the hips as spinal compression progresses. [8] Some individuals experience **unexplained back discomfort or fatigue when standing upright**, reflecting weakening spinal support structures. [9] As these structural changes progress, there may also be **subtle changes in gait**, including slower walking speed, shorter steps, or a slightly forward-flexed posture, often related to balance changes and altered biomechanics rather than bone loss alone. [8,10]

In addition to these changes, other subtle signs may develop. Individuals may notice a **weaker grip strength** or difficulty opening jars, along with a **gradual decline in strength or stamina** during everyday activities. [10] There may be **receding gums**, which can be associated with jawbone loss, [11] or brittle, easily breaking nails that may reflect underlying mineral imbalances. [10] Some people experience **changes in balance or increased unsteadiness**, raising the risk of falls even before a fracture occurs. [8,9] Others report **persistent, low-grade aches in the hips, wrists, or back** without a clear injury. [9] Because these signs develop slowly, they are



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often mistaken for normal aging, but collectively they may indicate early bone loss and warrant further evaluation. [8] *Oh boy! Chalking it up to normal aging! Now I am seeing more reasons they are called the “Silent Diseases”. And the medical industry is turning a blind eye and waiting until it is almost too late to help our bodies heal and/or prevent these ‘osteos’ through good nutrition and lifestyle changes. We need to change this paradigm! (I’m stepping down from my soapbox so we can continue. 😊)*

How Is It Diagnosed?

Diagnosis of low bone density begins with a **DEXA (Dual-Energy X-ray Absorptiometry) scan**, the gold standard for identifying both osteopenia and osteoporosis. This painless, low-radiation test measures bone mineral density at key sites such as the lumbar spine, hip (pelvis), and sometimes the forearm, allowing providers to detect bone loss before fractures occur. Diagnosis is based on comparing your bone density to standardized reference values, along with consideration of personal risk factors, medical history, and, in some cases, fracture history. [5,6]

A DEXA scan works by using two low-dose X-ray beams to calculate bone density and generate a **T-score**, which compares your bone density to that of a healthy young adult. A T-score of **-1.0 or higher is normal**, **-1.0 to -2.5 indicates osteopenia (early bone loss)**, and **-2.5 or lower confirms osteoporosis**, reflecting a significantly increased fracture risk. Screening is generally recommended for **women beginning at age 65 and men at age 70**, or earlier (around age 50) if risk factors are present, and is typically repeated **every 1–2 years** depending on baseline results and treatment status. Earlier or more frequent testing is appropriate for individuals with a **history of fractures after age 50, long-term steroid use, thyroid disorders, low body weight (e.g., under 125 lbs), smoking, excessive alcohol use, malabsorption conditions such as Celiac disease, early menopause, or a family history of osteoporosis**, as well as those taking medications known to negatively impact bone density. [5,6,7]

The Functional Nutrition Approach

Looking at osteopenia and osteoporosis through a functional lens, we want to look at diet and lifestyle changes in order to age gracefully and be able to pursue our retirement dreams. As we have already discovered, they are ‘silent diseases’ caused by aging, hormonal changes (especially post-menopause), poor diet, gut dysfunction, smoking, excess alcohol, sedentary lifestyle and/or certain medications. These ‘osteos’ did not occur overnight but over a lifetime (our lifetime). We need to remember, *‘One size does not fit all’* and there is no *‘quick fix’*! And we need keep in mind the mantra:

**Everything is connected,
We are all unique,
All things matter.**

What Can We Do?

Good question! Let’s take a look... The best recommendation is to ‘get back to the basics’... basic nutrition! The simple answer is:



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- 1) **drinking plenty of water,**
- 2) **eating real whole foods,** and
- 3) **making the appropriate lifestyle changes.**

These lifestyle changes include:

- Limit excess caffeine
- Avoid smoking and excessive alcohol consumption
- Avoid high processed foods and eating more whole foods
- Reduce chronic stress: High cortisol levels will break down bones.
- Go Soak Up Some Sun: Sunshine also helps regulate circadian rhythm and boosts Vitamin D3 production. It is recommended to get 10-30 minutes midday sun (depending on skin type, the more melanin in the skin the more time needed) provides adequate production of Vitamin D3 in your body.
- Get enough sleep: The average adult needs **7 – 9 hours of quality sleep** per night. **Bone repair happens at night.**
- Exercising – When exercising for bone health, it is recommended to focus less on a strenuous workout and more on resistance training and weight-bearing exercise, where the bones and muscles work against gravity as the feet and legs bear the body's weight. The key is to gently load your bones to strengthen them. ([more on light exercise below](#))

So, Let's Talk Nutrition...

We have already talked about some of the causes of osteopenia and osteoporosis with one of the common root causes being nutritional deficiencies. [Let's take look at the nutrients required for healthy bones...](#) The table below lists the key nutrients that work together for optimal bone health (Please note the listed dosage from all sources.):

Nutrient	Amount	Food Sources	Notes
Magnesium	400–800 mg*	Pumpkin seeds, chia seeds, nuts (almonds and cashews), dark chocolate, leafy greens, legumes	For collagen production and the soft tissue matrix. *Research suggests that for bone health and preventing bone density loss (osteopenia), a total daily intake of 400–800 mg is optimal.
Calcium	1,000-1,200 mg (not to exceed 390 mg @ 1 time)	Sardines, salmon, leafy greens, almonds, dairy products, soy products (if tolerated)	For mineralization. Daily intake of calcium should not exceed 2,000mg as excess calcium may cause kidney stones and cardio issues. Vitamin K2 needs to be taken with calcium to prevent the calcium from being deposited in the arteries instead of the bones!



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Nutrient	Amount	Food Sources	Notes
Vitamin K2	90-120mg	Natto, grass-fed dairy, egg yolks	A fat-soluble vitamin for directing calcium from arteries to the bones. Lowers fracture risk.
Vitamin D3	1000-2000 IU	Sunlight, cod liver oil, fatty fish (salmon, mackerel, rainbow trout, herring), egg yolks	For bone flexibility and resilience; A fat soluble vitamin to be taken with food – especially healthy fat.
Vitamin C	1,000 mg/day	Citrus fruits, red peppers, broccoli, and papaya	To produce collagen; Should be combined with Vitamin E. Vitamin C has shown potential to prevent bone loss in the spine similarly to resistance exercise. While dietary intake is safe, supplements should generally not exceed 2,000 mg/day , as higher doses can cause side effects like gastrointestinal discomfort.
Protein	Depends on body weight, age and activity level, but generally consume 1.0 to 1.2 grams of protein per kilogram of body weight. **	Eggs, fish, poultry, legumes, lean meat, dairy, soy, nuts	Protein makes up roughly 50% of bone volume and 30% of bone mass. It also helps with muscle loss (sarcopenia) - gives muscles needed fuel. <i>** (for those who don't want to do the math...) Protein intake of 25–30g protein per meal appears to maximally stimulate muscle protein synthesis in older adults. [14]</i>
Collagen (bioactive collagen peptides)	5 grams (1/3 Tbls)	Bone broth, fish, organ meats, gelatin, shellfish and egg whites.	To create the flexible framework. Shown to increase bone mineral density in the lumbar spine and femoral neck. Collagen works best in conjunction with calcium and Vitamin D to address the 50% collagen and 50% calcium composition of bone.

The above listed nutrients are crucial for bone health maintenance and to help prevent progression to osteoporosis. It is also important to keep the following in mind:

- Adequate intake of **calcium** and **vitamin D** can help mitigate some aspects of osteo-sarcopenia (low bone mass/density combined with low muscle mass, strength and function), **such as lean mass and bone density loss.**



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- For maximum absorption, it is often recommended to space **Calcium** and **Magnesium 2 to 4 hours apart**; Maintaining an appropriate ratio (often suggested as **1:1 or 2:1** calcium to magnesium) is important, as high calcium concentrations can decrease the body's ability to absorb magnesium.
- For optimal absorption, **calcium citrate** can be taken with or without food, while **calcium carbonate** should be taken with meals.
- **Calcium citrate** is preferred over **calcium carbonate**, especially for patients on proton pump inhibitors (PPI), because citrate does not require gastric acid for absorption.

For advanced support consider including:

- Trace minerals such as **boron, zinc**, and **silica**.
- **Omega-3** (reduce inflammation)
- **Genistein** -Found in soy products like tempeh and tofu, also in legumes in smaller amounts is a Phyto-estrogen (meaning plant that acts like estrogen) Genistein increases bone mineral density by decreasing bone breakdown. Before taking genistein, check with your physician as whether it will compete with any Bioidentical Hormone Replacement Therapy (BHRT) you are taking.

Resistance Is Futile? (Resistance Training That Is!)

Wrong! Resistance and weight-bearing exercise is necessary for optimal bone health! Your bones and muscles need gentle load and resistance to properly regenerate and build. Keep in mind, along with healthy eating and lifestyle changes, regular **weight-bearing exercise, resistance training, Tai-Chi, swimming/water aerobics, walking/hiking, dancing and light impact jumping** (if appropriate) are important complementary measures for maintaining bone health. Our bones not only respond to motion but also respond to load and resistance to keep them healthy and strong. For mild osteopenia, the goal is to simply and **gently load your bones** so they get stronger—without risking injury.

Please note: Before a person with osteoporosis initiates a new vigorous exercise program, such as running or heavy weight lifting, a doctor's evaluation is deemed appropriate.

What About a Weighted Vest?

I am glad you asked! A **weighted vest can be helpful** for mild osteopenia—but it's not where you start. They are especially useful for: Walking, Step-ups and Squats. A weighted vest adds a gentle load to your bones, which can:

- Stimulate bone strength (especially hips and spine)
- Make simple exercises more effective
- Improve balance and stability over time

However, avoid starting with a vest if:

- You are just beginning resistance training,
- You have balance issues,
- You feel unsteady during basic exercises (see exercises below),
- You haven't built a strength foundation yet.



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Think of a weighted vest as a **progression tool**, not a beginner tool. *So, you may be asking... When will I be ready to wear a weighted vest? Good question!* You are ready for the weighted vest when you have been consistent in your 'basic unweighted' workout for 4-8 weeks, the exercises feel "easy", and you have good balance and control. To start safely, go very light with a 2- 5 lbs. total weight, enough to stimulate bone – Remember more IS NOT better! Use the weights for simple movements such as walking, sit-to-stand, and step-up; avoiding twisting movements, or fast or jerky motion. You want to start with short durations of 10 – 15 minutes at first and slowly increase the time.

Why this works... Bones respond to **consistent, gentle load over time**. Your bones DO NOT need heavy weights and intense workouts to be strengthened. Think of it this way: 'Bones need progressive, steady stimulus!!'

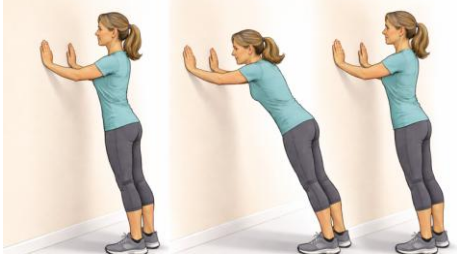
Sign me up!! Where do I start? You can also do simple exercises at home that DO NOT cost you much (no gym memberships - *where you feel like a piece of meat on an elliptical*, or a weighted vest that is too heavy for starters!) using items you can find around your home. And your bones do not need complicated workouts for strengthening. The **simplest, safest resistance exercises** are basic, functional movements using your body weight for light resistance. *Now that makes me smile!! 😊*

You May Be Thinking... What About a Personal Trainer?

Let's take a look... If you are a 'newbie' to resistance training or weight-bearing exercises and/or are drastically making lifestyle changes, it will behoove you to consider working with a qualified personal trainer—ideally one experienced in bone health or corrective exercise. This can be extremely valuable when addressing osteopenia or osteoporosis. A personal trainer can assess posture, balance, and strength, then design a safe, progressive program focused on proper resistance and weight-bearing exercises that stimulate bone growth without increasing fracture risk. They also ensure correct form, help avoid high-risk movements (like excessive spinal flexion or twisting), and build confidence so individuals can train effectively and safely over time.

Below are included a few very simple exercises that are focused on resistance and weight-bearing training (*Yawn, I am bored just looking at them, but I need to do them.*), which are widely recommended to maintain or improve bone density by applying mechanical load to the skeleton, stimulating bone formation and slowing bone loss in individuals with osteopenia or osteoporosis. **NOTE: Individuals should consult with a qualified healthcare provider before beginning any new exercise program to ensure safety and appropriateness based on their personal health history and risk factors.**



Exercise Visual	Discussion
Shoulder Rolls  1. UP: Lift your shoulders up toward your ears. 2. BACK: Roll your shoulders back, down, and around. 3. REPEAT: Continue the movement for 8-10 reps. Then reverse direction and repeat.	Warm Up • Lift shoulders up towards ears • Roll shoulders back down and around • Move slowly and with control; breathing deeply and focusing on the sensation in your shoulders. Do: 10 forwards; 10 backward Safety Cues: Avoid quick/jerky moves; breathe steadily; relax neck and jaw; use comfortable range of motion. Benefit: Excellent warm-up that's gentle, low-risk, and help prepare the upper body for posture and resistance exercises without placing excessive stress on the spine.
1. Sit-to-Stand (Chair Squat) 	Mimics real life + strengthens hips/spine • Sit in a chair → stand up → sit back down • Keep chest upright • Use hands if needed at first Do: 8–12 reps, 2-3 sets, 2–3 times per week (with a rest day in between) Safety Cues: Keep chest upright; push hips back; knees track over toes; use a chair for support if needed Benefit: Squats load the hips, spine, and legs, stimulating bone formation while improving lower body strength and stability necessary for daily activities. [12,13]
2(a). Wall Push-Ups 	Upper body + spine support • Stand facing a wall • Hands on wall → lower chest → push back Do: 8–12 reps, 2-3 sets, 2–3 times per week (with a rest day in between) Safety Cues: Keep body in a straight line; avoid arching the low back; move slowly and with control; stop if shoulder or wrist pain occurs. Benefit: Easier than floor push-ups. This gentle, weight-bearing exercise helps stimulate bone formation in the wrists, arms, and upper spine by applying safe resistance through the upper body. It also improves upper-body strength, posture, and joint stability, which can help reduce fall risk and support overall bone health in individuals with osteopenia or osteoporosis. [12,13]



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Exercise Visual	Discussion
2(b). Counter Push-up (Kitchen Counter) 	Upper body + spine support (Progression from Wall Push-Ups) - Stand facing a wall. Place hands shoulder width apart on counter. Step back and create a straight line from head to heel. Engage core and keep neck neutral. - Bend elbows and slowly lower chest toward counter, keeping elbows at ~45 deg angle to body. Stop when chest is a few inches from counter. - Push back up pressing thru palms of hands to straighten arms. Return to start position. Do: 8-12 reps, 2-3 sets, 2–3 times per week (with a rest day in between) Safety Cues: Maintain neutral spine; keep elbows at a slight angle (not flared); engage core; avoid dropping hips Benefit: Performing push-ups against a kitchen counter provides similar benefits as the Wall Push-Ups with slightly greater resistance, placing more load through the upper extremities to further support bone density, muscular strength, and stability. This progression can enhance functional strength and balance, while still remaining safe and accessible for individuals building bone health. [12,13]
3. Heel Raises/Drops 	Strengthens lower legs + improves balance - Stand holding a chair - Lift heels → slowly lower Do: 2–3 sets of 10–15 reps, Daily or 3–5x/week (as tolerated) Safety Cues: Keep movements gentle; avoid if high fracture risk; hold support for balance; land softly Benefit: This mild impact exercise helps stimulate bone growth in the hips and spine by introducing safe, brief loading forces, which are known to promote bone remodeling. [12,13]
4. Wall Angels (Wall Slides) 	Posture + upper back (important for spine health) - Stand with your back against a wall - Keep head, upper back, and hips touching the wall - Raise arms to a “goalpost” position - Slowly slide arms up and down the wall Do: 2–3 sets of 8-12 reps, 2–3x/week Safety Cues: Keep head, upper back, and hips against the wall; move slowly and with control; avoid arching the lower back; only move within a pain-free range.



Exercise Visual	Discussion
	Benefit: This exercise improves posture, upper back strength, and shoulder mobility and stability. Wall angels activate the upper back muscles (rhomboids, mid-traps). This posture-focused exercise strengthens the upper back and shoulders while promoting proper spinal alignment, helping reduce fracture risk and support bone health in individuals with osteopenia or osteoporosis. By encouraging scapular retraction and improving thoracic extension, it helps counteract forward-rounded posture commonly associated with bone loss and encourage proper spinal alignment. [12,13]
5. Step-Ups (Low Step) 	Builds hip and leg bone strength • Step onto a low stair or platform • Step down slowly Do: 2–3 sets of 8–10 reps per leg, 2–3 times per week Safety Cues: Use a stable surface; step up with control; avoid pushing off the back foot; hold a rail or wall if needed; Knee should stay below or around hip height when stepping up. Benefit: This weight-bearing exercise targets the hips and legs, helping build bone density in the lower body while improving balance, coordination, and functional strength. [12,13] Recommended step heights: • Beginner: 4–6 inches (low stair, aerobic step, or sturdy platform) • Progression: 6–8 inches as strength and balance improve • Advanced (if appropriate): Up to ~10–12 inches <i>only if stable and pain-free</i> NOTE: You don't need a high step to get bone benefits—proper loading and control matter more than height.
6. Standing Marches (Balance Exercise) 	Hip Stability, Improve Balance • Beginner: Hold chair or counter for support • Intermediate: Use fingertips on wall or counter for balance • Advanced: March without support For each level, March lifting one knee to hip height; lower with control. Do: 2–3 sets of 10–20 marches, Daily or 3–5x/week Safety Cues: Stand tall; engage core; use support based on level above; move slowly and with control.



Exercise Visual	Discussion
	Benefit: This low-impact movement improves balance, coordination, and hip stability, helping reduce fall risk —an essential component of fracture prevention. [12,13]

What Not to Do... Movements and Activities to Avoid with Osteopenia and Osteoporosis

Individuals with osteopenia or osteoporosis should avoid movements and activities that place excessive compression, flexion, or twisting forces on weakened bones—particularly the spine. Forward bending at the waist, twisting motions, jerky high-impact activities, and heavy lifting performed with poor body mechanics may increase the risk of vertebral compression fractures.[\[15\]](#) Examples include toe-touch stretches, sit-ups, crunches, forceful twisting exercises, and lifting heavy objects while bending forward.[\[15,16\]](#) High-impact activities such as jumping or uncontrolled running may also increase fracture risk in individuals with significant bone loss.[\[15\]](#) Instead, experts recommend controlled weight-bearing exercise, posture-focused movement, balance training, and proper lifting techniques using the hips and knees rather than rounding the spine.[\[15,17\]](#) **NOTE:** *(Not to sound like a broken record, but...)* y'all NEED to consult a qualified healthcare provider or physical therapist before beginning any exercise program, particularly if osteoporosis is advanced or fractures have already occurred.[\[15\]](#)

Concluding Remarks

Receiving a diagnosis of osteopenia or osteoporosis can feel overwhelming, but osteopenia should be viewed as an early warning sign—not a sentence of inevitable decline. The encouraging news is that our bones respond to consistent support through proper nutrition, movement, sleep, stress management, and healthy lifestyle choices.

Simple daily habits such as eating nutrient-dense whole foods, getting adequate protein and minerals, engaging in safe resistance and weight-bearing exercise, improving sleep, and reducing inflammation can collectively support stronger bones, better balance, improved posture, and overall quality of life. Small, consistent changes truly matter.

From a Functional Nutrition perspective, bone health does not exist in isolation. Hormones, digestion, inflammation, nutrient absorption, muscle health, stress, and movement patterns are all connected. Because every person is uniquely created, there is no “one-size-fits-all” solution. The goal is not simply improving a DEXA score, but supporting the body as a whole to promote strength, resilience, mobility, and healthy aging.

Most importantly, it is never too early—or too late—to begin caring for your bones. Proactive steps taken today may help preserve independence, reduce fracture risk, and support a healthier future.



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As believers, we are reminded that caring for our bodies is an act of stewardship:

“Whether therefore ye eat, or drink, or whatsoever ye do, do all to the glory of God.”

— 1 COR 10:31 (KJV)

Reference material available upon request.

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Tracey Williams is not a medical doctor. She is a Certified Functional Nutrition Counselor/Researcher. Tracey Williams does not diagnose or treat any medical illness or condition.



Tracey Williams, CFNC

At **Basic Biblical Nutrition**, we believe that **it IS personal** — because everything is connected, every person is uniquely created, and all things matter. We approach Functional Nutrition through a biblical lens, continually asking, “What does God say?” as we pursue health that glorifies Him in body, mind, and spirit. As Scripture reminds us:

“Whether therefore ye eat, or drink, or whatsoever ye do, do all to the glory of God.”
(1 Cor 10:31, KJV)

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